

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Phase 1, Open-Label, Dose Finding Study of CC-95251 Alone and in Combination with Antineoplastic Agents in Subjects with Acute Myeloid Leukemia and Myelodysplastic Syndromes,

Short Title/Acronym: A Study to Assess the Effect of CC-95251 in Participants With Acute Myeloid Leukemia and Myelodysplastic Syndromes,

Protocol Number: CA059-001,

NCT Number: NCT05168202,

Posting ID: 975256704522898293 **Sponsor/Company Name:** Bristol-Myers Squibb (BMS) / Celgene Corporation,

Investigational Product: CC-95251 (anzurstobart / BMS-986351) alone, and in combination with Azacitidine and Venetoclax,,

Phase of Development: Phase 1,

Trial Status: TERMINATED (Study was terminated early) **Medical**

Monitor: GSM-CT Representative (BMS Clinical Trials)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and preliminary clinical activity of CC-95251 alone and in combination with antineoplastic agents in participants with relapsed or refractory Acute Myeloid Leukemia (AML) and relapsed or refractory and treatment-naïve higher risk Myelodysplastic Syndromes (MDS). **Secondary Objectives:** To evaluate the pharmacokinetics (C_{max}, T_{max}, AUC(0-T)) and efficacy of CC-95251 alone and in combination regimens.

2.2 Background & Rationale To address the need for novel targeted therapies in AML and MDS. CC-95251 is a monoclonal antibody that targets the CD47-SIRP α innate immune checkpoint. By blocking the SIRP α receptor on macrophages, it prevents cancer cells from evading the immune system and potentiates macrophage-mediated tumor phagocytosis, thereby targeting leukemia/dysplastic cells for removal from the body,,.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional, **Control Type:** Single-arm/Parallel Assignment (Dose Escalation/Expansion)

Blinding: Open-label, **Randomization:** Non-Randomized

3.2 Planned Enrollment & Duration Target N: 56 participants

Number of Sites: Multicenter (Global, including UK sites)

Estimated Study Start: 19-Jan-2022 **Estimated Primary Completion:** 30-Jul-2024 **Current Status:** Terminated

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years with an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 to 2. 2. For Parts A & B: Relapsed or refractory (R/R) Acute Myeloid Leukemia (AML) or R/R Myelodysplastic Syndromes (MDS) as defined by the 2016 WHO Classification with intermediate, high or very high risk by IPSS-R. 3. For Part C: Treatment-naïve (TN) MDS as defined by the 2016 WHO Classification with intermediate, high or very high risk by IPSS-R. 4. For Part D: Treatment-naïve (TN) AML as defined by the 2016 WHO Classification, including secondary AML and therapy-related AML in participants who are ineligible for intensive chemotherapy and allogeneic HSCT.

4.2 Exclusion Criteria 1. Acute promyelocytic leukemia,. 2. Immediately life-threatening, severe complications of leukemia (e.g., disseminated/uncontrolled infection, uncontrolled bleeding, and/or uncontrolled disseminated intravascular coagulation),. 3. Prior treatment with a CD47 or SIRP α targeting agent, or currently on chronic systemic immunosuppressive therapy/corticosteroids,. 4. Prior systemic cancer-directed treatments or investigational modalities $\leq$ 5 half-lives or 4 weeks prior to starting study treatment, whichever is shorter (relapsed or refractory participants only). 5. Pregnant or nursing participants, or any active/uncontrolled infection placing the participant at unacceptable risk.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: CC-95251 given at specified dose-escalation levels on specific days per cycle. Administered as monotherapy or in combination with standard-of-care azacitidine (Trade names: Vidaza, Onureg; ATC Code: L01BC07) with/without venetoclax,. **Route of Administration:** Intravenous (IV) infusion (may require hospitalisation for at least 24 hours initially). **Duration of Treatment:** Up to a maximum of 2 years, or until disease worsens, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Azacitidine and Venetoclax according to combination arm assignments. **Prohibited:** CD47 or SIRP α targeting agents; chronic systemic immunosuppressive therapy or corticosteroids,.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Up to Day -1 | Informed Consent, Medical History, Physical Exams, ECG, Heart Scans (MUGA/ECHO), Labs | | **Baseline** | Day 1 | Enrolment, First Dose Administration, Vital Signs | | **Interim** | Cycle 1 to End of Treatment | Safety/ DLT Assessments (Days 1-28/42), PK Sampling (e.g., C1D1, C1D22, C2D22), Efficacy Scans, | | **End of Study** | Follow-up | Safety follow-up up to 56 days post last dose; Long-term follow-up every 8-12 weeks for up to 2 years |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **Number of Participants With DLTs:** A dose-limiting toxicity (DLT) is defined as any treatment-related toxicity during Cycle 1 (Days 1-28, up to 42) not clearly due to illness or external causes (including Grade ≥ 3 non-hematologic toxicity, confirmed Hy's law case, specific severe hematologic toxicities/prolonged myelosuppression, and AEs requiring dose reduction). 2. **Number of Participants With Treatment Emergent Adverse Events (TEAEs):** Any noxious, unintended, or untoward medical occurrence that may appear or worsen in a participant during the course of the study. 3. **Number of Participants With Grade 3 or Higher Laboratory Abnormalities:** Clinical laboratory values graded according to CTCAE Version 5.

7.2 Secondary Outcomes 1. Maximum plasma concentration of drug (C_{max}). 2. Time to maximum plasma concentration (T_{max}). 3. Area under the curve (AUC(0-T)).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous evaluation of physical exams, vital signs, ECG, and laboratory data. Assessed via changes in ECOG performance status and recorded as AEs,. **Serious Adverse Events (SAEs):** Protocol-mandated rapid reporting timelines, with special attention to DLTs occurring during Cycle 1 of dose escalation,. **Data Monitoring Committee (DMC):** Internal safety review committee evaluates dose-escalation safety clearance before proceeding to the next dose cohort.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Population (all participants who receive at least one dose of the investigational product). **Sample Size Justification:** Target $N = 56$ participants planned across parts A, B, C, and D, providing adequate powering for Phase 1 dose-escalation rules (MTD determination) and subsequent dose-

expansion signal seeking. **Handling of Missing Data:** Standard Phase 1 algorithms for missing data handling; baseline values defined as the last non-missing value on or before the first dose.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular clinical site monitoring visits to verify protocol compliance, AE reporting, and IMP accountability.

Audit Trail: Robust electronic case report form (eCRF) data capture with source data verification (SDV).

11. Study Identifiers & Governance

Primary ID: CA059-001 **Registry Number:** NCT05168202 **Posting ID:** 975256704522898293 **Sponsor/Lead Organization:** Bristol-Myers Squibb / Celgene Corporation **Study Title:** A Study to Assess the Effect of CC-95251 in Participants With Acute Myeloid Leukemia and Myelodysplastic Syndromes **Official Regulatory Title:** A Phase 1, Open-Label, Dose Finding Study of CC-95251 Alone and in Combination with Antineoplastic Agents in Subjects with Acute Myeloid Leukemia and Myelodysplastic Syndromes

12. Clinical Framework (AML/MDS Specific)

Disease Areas: Leukemia, Myeloid, Acute, Myelodysplastic Syndromes **Preferred UMLS Name:** Myeloid Neoplasms **Semantic Type:** Neoplastic Process (SNOMED ID: 93143009, MeSH Code: D007938) **Diagnosis Threshold:** 2016 WHO Classification Criteria with IPSS-R risk stratification **Medical Methodology:** Anti-SIRP α monoclonal antibody therapy (CC-95251) $\pm$ antineoplastic agents (azacitidine/venetoclax) **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose

13. Study Procedures & Methodology

Management Pathway: Phase 1 open-label dose-escalation and expansion **Education Protocol (HCP):** Investigator site training for DLT reporting and combination dosing **Education Protocol (Patient):** Informed consent detailing IV infusion requirements and potential 24-hour hospitalization **Quality Audit Mechanism:** Safety Review Committee (SRC) oversight for cohort escalation

14. Observation & Follow-Up Schedule

Total Duration: Up to 2 years of active treatment + approx. 29 months total observation timeframe **Onsite Visit Cadence:** Pre-dose Baseline, Cycle 1 (Days 1, 22), Cycle 2 (Day 22), and ongoing per protocol cycles **Remote Monitoring Frequency:** Long-term follow-up every 8 weeks for Year 1, then every 12 weeks **Checklist Review Interval:** Continuous monitoring of ECOG score and AEs prior to each dosing cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				